



Pilot testing of the “First You Should Get Stronger” program among caregivers of older adults with dementia



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ABSTRACT

Objective: In this study, randomized controlled interventional study pattern was used to examine the effects of the “First You Should Get Stronger” program on the caregiving burden and healthy life style behavior of caregivers of dementia patients.

Methods: “Zarit Caregiver Burden Scale” and “Healthy Life Style Behavior Scale” were used. The study was completed with 40 caregivers in total with 20 in the intervention group and 20 in the control group. A statistically significant difference was determined between the “Zarit Caregiving Burden Scale” and “Healthy Life Style Behavior Scale” score averages of the intervention group that participated in the “First You Should Get Stronger” program in comparison with those of the control group.

Results: It is important for the healths of caregivers to include similar programs for the caregivers of dementia patients in continuous and regular applications.

Discussion: The results highlight the importance of the “First You Should Get Stronger” program significantly decreased the caregiving burden and significantly developed the healthy lifestyle behaviors of caregivers in the intervention group. Since dementia is a difficult neurological syndrome with patients cared at home, it generally wears out the caregivers significantly. It is suggested that the nurses and healthcare professionals working with dementia patients are evaluated separately and that they carry out caregiving applications within the scope of the “First You Should Get Stronger” program.

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1. Introduction

Dementia is a disease for which various related issues such as its care, treatment and the safety of the individuals should be considered carefully due to its negative effects on life and the quality of life (Mace & Rabins, 2012). Dementia is an important problem for societies because of its increasing prevalence, the unprofessional caregiving burden on the families as well as the economical burden it brings on national healthcare (Nichols, Martindale-Adams, Burns, Graney, & Zuber, 2011; Papastavrou, Kalokerinou, Papacostas, Tsangari, & Sourtzi, 2007). The prevalence of dementia increases with age and it is stated to be 5% for individuals above the age of 65 and 20% for individuals around the age of 80 (Ganguli, Dodge, Shen, Pandav, & DeKosky, 2005; Gaugler, Mittelman, Hepburn, & Newcomer, 2010). Caregiving for dementia is difficult and the level of burden for the caregiver is quite high (Collins & Swartz, 2011). Due to its progressive tendency, dementia is a process that affects many different aspects of the family

members and the caregiver since patients experience continuous changes (Ganguli et al., 2005). The patients are generally cared for in their homes by their family members. Providing care for a person with dementia in the community commonly places stress on the caregiver (Collins & Swartz, 2011; Dang, Badiye, & Kelkar, 2008). Caregivers who take on the role of giving care experience social, emotional, economic and physical burdens when they cannot cope with the difficulties they experience. Hence, it is quite important to support the caregivers via home visits by nurses (Roach, Keady, Bee, & Hope, 2008).

Various intervention programs are applied for caregivers of dementia patients. Interventions such as psychoeducation, technology-based multi-component psychosocial intervention, psychotherapy, cognitive behavioral therapy, mindfulness based intervention; case management and consultancy are carried out frequently for caregivers (Arango-Lasprilla et al., 2014; Czaja, Loewenstein, Schulz, Nair, & Perdomo, 2013; Ducharme, Lachance, Lévesque, Zarit, & Kergoat, 2015; Hou et al., 2013; Klein, Pendergrass, Becker, Hautzinger, & Pfeiffer, 2015). Many different interventions aiming to alleviate the negative consequences of providing care to a patient with dementia have been designed and

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implemented in the past 20 years. It was determined as a result of the studies carried out that the interventions aimed at caregivers of dementia patients are important and effective for caregivers (Brodaty, Green, & Koschera, 2003; Ducharme et al., 2011; Klein et al., 2015; Schulz et al., 2002). Pinquar and Sorensen carried out a study in (2006) during which it was put forth that the intervention programs applied on caregivers of dementia patients had statistically significant effects on caregivers but that they had minor effects on the burden, depression of the caregiver as well as the well-being ability/knowledge and symptoms of care recipient.

It is also put forth that psychoeducation interventions with active participation have wide ranging positive effects on caregivers (Pinquar & Sörensen, 2006). Jensen et al. carried out a metaanalysis study on the education intervention programs aimed at caregivers of dementia patients in which it was determined that educational programs have a moderate effect on caregiver burden and a small effect on depression (Jensen, Agbata, Canavan, & McCarthy, 2015). Despite these positive effects, there is no systematic, planned and continuous intervention program applied for the caregivers of dementia patients in order to help them cope with the difficulties during the caregiving process (Brodaty & Donkin, 2009). Research suggests the need to develop and implement effective intervention strategies for this population (Jensen et al., 2015; Pinquar and Sörensen, 2006). There is no systematic or planned intervention program applied on the caregivers of dementia patients in Turkey. The objective of this study which started as a result of this requirement was to evaluate the effectiveness of the **“First You Should Get Stronger”** program that might be applied on the caregivers of individuals with dementia on the caregiving burden and healthy life style behaviors.

1.1. Study hypothesis

Hypothesis 1. There will be a statistically significant difference between the life style behavior score averages of the caregivers in the **“First You Should Get Stronger”** group before and after the program and the life style behavior score averages of the caregivers in the control group.

Hypothesis 2. The Zarit Caregiving Burden scale score averages of the caregivers in the **“First You Should Get Stronger”** group before and after the program will be greater in comparison with the score averages of the caregivers in the control group.

2. Material and methods

2.1. Design

The study is an experimental type randomized controlled study designed as a pre-test post-test model. Caregivers to be assigned to the intervention and control groups were selected from among those registered at the Erenköy Rotary Club Family Health Center who provides care to patients diagnosed with dementia. The pre-test post-test data of the study and the application stage of the program were carried out at the homes of the caregivers in the form of home visits for a period of seven weeks. Visits were carried out by the researcher once every week with each session lasting 45 min.

2.2. Sample

The population of the study consisted of 40 caregivers registered at the Erenköy Rotary Club Family Health Center who are also the first degree relatives of individuals diagnosed with dementia according to DSM V. The selection of the individuals who

participated in the study was carried out at the Family Health Center. A list of dementia patients was obtained from the family health center after which the experiment and control groups were determined via simple randomization method. The caregivers who agreed to participate in the study were classified into two groups of 20 people as intervention and control groups via full randomization method without carrying out sample size calculation. Study inclusion criteria for the caregivers were selected as acceptance to participate in the study, literacy and being the first degree relative of an individual diagnosed with dementia according to DSM V at the polyclinic. Exclusion criteria were determined as failure of the caregiver to visit the patient for 2 or more times.

2.3. Measurements

The information form developed by the researchers for evaluating the sociodemographic properties of the individuals in the intervention and control groups, the Zarit Caregiving Burden Scale that evaluates the burden, health, psychological well-being, economy and social life of the caregiver of the dementia patient along with the Healthy Life Style Behavior Scale II which evaluates the healthy life style of the caregiver were used as data acquisition tools in the study. Pre-test and post-test data were acquired in the home environment by the researcher using face-to-face interviews and home visits methods.

Sociodemographic Information Form: The form developed by the researcher based on literature consists of questions for determining the sociodemographic properties of the caregiver as well as the disease properties of the individual who is being cared for.

Healthy Life Style Behavior Scale II: The scale was developed by Walker and was revised in 1996. The scale measures behavior that develops one's health in relation with the healthy life style of the individual. The scale consists of 52 items and 6 sub-factors. The sub-factors are moral development, health responsibility, feeding, interpersonal relations and stress management. The general score of the scale provides the healthy life style behavior score. All items of the scale are positive. The scaling is 4-point Likert type. The answers are accepted as Never (1), sometimes (2), frequently (3), regularly (4). The lowest score for the whole scale is 52, whereas the highest score is 208. The Alpha reliability coefficient of the scale is 0.94. The Alpha coefficient reliability values of the sub-factors of the scale vary between 0.79–0.87. The Turkish validity and reliability study was carried out by Bahar, Beşer, Gordes, Ersin, and Kissal 2008. The Cronbach Alpha coefficient of the Healthy Life Style Behavior Scale II is 0.92 and it has a high level of reliability (Bahar et al., 2008). It was observed that the lowest Cronbach Alpha reliability coefficient for the subscales of the Healthy Life Style Behavior Scale II used in the study was 0,76 and that the highest was 0,90 for the data obtained for the pre-test and post-test results for the intervention and control groups.

Zarit Caregiving Burden Scale: The scale was developed by Zarit, Reever and Bach-Peterson in 1980 and it is used to evaluate the stress levels experienced by the caregivers of individuals in need or elderly people. The scale can be filled by the caregiver or the researcher can ask the questions which consist of 22 statements that determine the effects of caregiving on the life of the individual. The scale has a Likert type evaluation with answers ranging from “0” to “4” in the form of never, rarely, sometimes, frequently or almost always (Zarit & Zarit, 1990). The minimum score that can be acquired from the scale is 0, whereas the maximum score is 88. The items in the scale are generally related with the social and emotional area and high scores indicate that the difficulties experienced are intense (Inci, 2006; Zarit & Zarit, 1990). The reliability study for the Caregiving Burden Scale was carried out by Inci and Erdem (2008) using language equivalence,

content validity and construct validity methods whereas internal consistency, item analysis test-retest reliability methods were used for the reliability study. The internal consistency coefficient of the scale was found to vary between 0.87 and 0.94, whereas the test-retest reliability was determined to be 0.71. It was observed that the lowest Cronbach Alpha reliability coefficient was 0.81 and that the highest was 0.93 for the pre-test and post-test data obtained for the Zarit Caregiving Burden Scale in the intervention and control groups.

2.4. Intervention

The content of the program was designed in accordance with the relevant literature before the study was implemented (Alves, Teixeira, Azevedo, Duarte, & Paúl, 2015; Arango-Lasprilla et al., 2014; Ducharme et al., 2015; Hou et al., 2013; Pinquar and Sörensen, 2006). We sought expert opinions from four faculty members in the psychiatric nursing department (1), public health nursing department (2), internal medical nursing department (1) who were specialized in working with the families of patients with dementia. Final revisions were made in the program in accordance with the opinions of experts. The program consists of seven sessions for strengthening the caregiver, decreasing the caregiving burden and developing healthy life style behaviors (Table 1).

2.5. Study variables

The caregiving burden score average and healthy life style behavior scale sub dimension score averages are dependent variables in the study. The “First You Should Get Stronger” program used in the study is an independent variable.

2.6. Data analysis

Data of the study were evaluated using SPSS for Windows 15.0 (Statistical Package for Social Science) statistical package software. The total scores of individuals for each scale were calculated after the pre-test and post-test data were collected. First number and percent distributions were examined for evaluating the data after which the variables of the two groups were compared among each other and t-test was used for those that fit the normal distribution and Wilcoxon sign test for those that do not; whereas Mann Whitney U test was used for the intergroup comparison of the variables since it did not fit the normal distribution. Intergroup

homogeneity chi-square analysis was used for the comparison of the independent variables of both groups. Cronbach's Alpha coefficient will be calculated for the general reliability and the reliability of the sub-dimensions. The results were evaluated to be within the 95% reliability interval and $p < 0.05$ statistically significance level.

3. Ethical procedures

Ethical approval was obtained from the Antalya Education and Research Hospital Ethical Council in order to be able to carry out the study, whereas corporate approval was obtained from the Antalya Directorate of Public Health and Family Health Center. The name, objective, duration and type of the study were explained to all individuals assigned to the intervention and control groups and consent forms were read. Thus, it was ensured that they understood the objective and scope of the study. Written consents were taken from patients who accepted to participate in the study. Data acquisition and application were started after taking the consent of the participants. Details of the intervention were explained to the members of the intervention group.

4. Results

The distributions of the caregivers of individuals with dementia in the experiment and control group of the study with regard to their descriptive properties are given in Table 2. Socio-demographic properties of the caregivers in the experiment and control groups such as gender ($p = 1.00$), educational status ($p = 1.00$) and marital status ($p = 0.225$) as well as the gender ($p = 0.374$) and dependency status ($p = 0.376$) of the care receivers were compared via chi-square analysis and it was observed that there is no statistically significant difference between the groups (Table 2).

When the age and average caregiving times for the caregivers in the experiment and control groups were examined, it was observed that the age averages of the caregivers in both

groups were similar (Experiment: = 52.97; Control Group: = 51.76). The average caregiving time of the caregivers in the experiment group was 25.8 months, whereas that of the caregivers in the control group was 27.9 months. The age and caregiving time averages of the caregivers in the experiment and control groups were evaluated via Mann Whitney U test and no statistically significant difference was determined with regard to the age and

Table 1
“First You Should Get Stronger” Caregiving Program Sessions.

Session	Name of the Session	Goals
Session 1	Introduction, learning about dementia and the individual for whom care is given	• Caregiving burden • Healthy life style behaviors
Session 2	Communication with individuals with dementia (caregiving without being derogatory)	• Caregiving burden • Healthy life style behaviors
Session 3	Defining the difficulties experienced by the individual with dementia	• Caregiving burden • Healthy life style behaviors
Session 4	Carrying out the systematics of daily works	• Caregiving burden • Healthy life style behaviors
Session 5	Determining social support systems	• Caregiving burden • Healthy life style behaviors
Session 6	Carrying out activities for enhancing and developing the health of the caregiver	• Caregiving burden • Healthy life style behaviors
Session 7	General evaluation and closure	• Caregiving burden • Healthy life style behaviors

Table 2

Comparison of the Sociodemographic Properties of the Experiment and Control Groups.

	Experiment Group (n:20)		Control Group (n:20)		χ^2	p
	n	%	n	%		
Of the caregiver, Gender						
Female	14	70.0	13	65.0	0,739	1.00 ^b
Male	6	30,0	7	35.0		
Educational Status						
Literate	3	15.0	3	15.0	1.360	1.00 ^b
Primary School	11	55.0	14	70.0		
High School	6	30.0	3	15.0		
Marital Status						
Married	14	70.0	13	65.0	0,256	0,225 ^a
Widow/Divorces	6	30,0	7	35.0		
Of the Care Receiver; Gender						
Female	9	45.0	7	35.0	0,519	0,374 ^a
Male	11	55.0	13	65.0		
Dependency Status						
Semi-Dependent	10	50.0	12	60.0	0.525	0,376 ^a
Dependent	10	50.0	8	40.0		

^a Yates corrected chi-square analysis was carried out since the observed number was less than 25.^b Fisher Kesin test was carried out since the expected number was less than 5.

caregiving time period between the caregivers who participated in the study (Table 3).

Pre-test and post-test score averages for the sub-dimensions of the Healthy Life Style Behavior Scale were compared for caregivers in the experiment and control groups. The difference with respect to time between the Healthy Life Style Behavior Scale sub-dimensions (health responsibility, physical activity, feeding, moral development, interpersonal communication and stress management) of the experiment and control groups were evaluated via Wilcoxon sign test and t test. According to the analysis, a statistically significant difference ($p < 0.05$) was determined between all the sub-dimensions of the healthy life style behavior scale of the experiment group before and after the program, whereas no statistically significant difference was determined for the control group ($p > 0.05$) (Table 4).

The difference between the Healthy Life Style Behavior Scale sub-dimension (health responsibility, physical activity, feeding, moral development, interpersonal communication and stress management) score averages of the caregivers in the experiment and control groups were evaluated via Mann Whitney U test. Whereas no statistically significant difference was determined between the two groups prior to the program ($p > 0.05$), it was determined after the program that the score averages of the caregivers in the experiment group were higher at a statistically significant level in comparison with those in the control group ($p < 0.05$) (Table 4).

Caregiving burden scale pre-test and post-test score averages for the caregivers in the experiment and control groups were compared. The difference with time of the caregiving burden scale

Table 3

Comparison of the Age, Caregiving Time Period and the Age Averages of the Care Receivers in the Experiment and Control Groups.

Properties	Experiment Group $\bar{x} \pm SS$	Control Group $\bar{x} \pm SS$	U	p
Caregiver Age	52.97 $\pm$ 8.19	51.76 $\pm$ 6.46	163.0	0.31
Caregiving Time Period (Months)	25.8 $\pm$ 9.37	27.9 $\pm$ 7.63	180.5	0.59
Care Receiver Age	70.65 $\pm$ 15.93	74.16 $\pm$ 8.46	183.0	0.64

U: Mann Whitney U Test.

Table 4

Pre-test and Post-test Healthy Life Style Behavior Scale Score Average Comparisons for the Experiment and Control Groups.

Healthy Life Style Behaviors	Experiment Group $\bar{x} \pm SS$		Control Group $\bar{x} \pm SS$		Before the Program	After the Program
	Before the Program	After the Program	Before the Program	After the Program	Test value, p	Test value, p
Health Responsibility	18.75 $\pm$ 3.71	23.20 $\pm$ 2.01	17.30 $\pm$ 1.55	17.05 $\pm$ 2.13	U:10.00, p:0.12	U:13.00, p:0.01
	z: -12.30, p:0.00		t: -1.930, p:0.06			
Physical Activity	16.35 $\pm$ 3.06	24.20 $\pm$ 1.47	16.70 $\pm$ 1.34	15.07 $\pm$ 1.48	U:11.00, p:0.26	U:12.00, p:0.00
	z:18.45, p:0.02		t: -1.23, p:0.34			
Feeding	18.10 $\pm$ 3.09	24.75 $\pm$ 1.25	18.15 $\pm$ 1.18	17.09 $\pm$ 1.25	U:1.039, p:0.30	U:10.50, p:0.00
	z: -15.61, p:0.00		t:0.35, p:0.07			
Moral Development	18.15 $\pm$ 3.88	22.10 $\pm$ 1.02	17.05 $\pm$ 0.94	16.20 $\pm$ 1.05	U:0.161, p:0.87	U:13.00, p:0.00
	z: -12.03, p:0.04		t:3.44, p:0.72			
Interpersonal Communication	18.00 $\pm$ 3.92	23.75 $\pm$ 1.40	19.55 $\pm$ 1.35	18.65 $\pm$ 1.34	U:75.00, p:0.78	U:14.50, p:0.00
	z: -13.64, p:0.02		t:5.91, p:0.82			
Stress Management	16.25 $\pm$ 3.40	22.05 $\pm$ 1.53	16.85 $\pm$ 1.98	16.65 $\pm$ 1.34	U:22.00, p:0.26	U:11.00, p:0.00
	z:10.452, p:0.00		t:0.60, p:0.44			

z:Wilcoxon Sign test.

U:Mann Whitney U test.

score averages of the experiment and control groups was evaluated via Wilcoxon sign test. According to the analysis, a statistically significant difference was determined between the caregiving burden score averages of the experiment group before and after the program ($z: -5.434, p:0.00$), whereas no statistically significant difference was determined in the control group ($z: -1.437, p:0.165$) (Table 5).

The difference between the caregiving burden scale score averages in the experiment and control groups were evaluated via Mann Whitney U test. Whereas no statistically significant difference was determined between the two groups prior to the program ($U:25.00, p:0.56$), it was determined after the program that the caregiving burden score averages of the caregivers in the experiment group were lower in comparison with those of the control group and that the difference was statistically significant ($U:40.00, p:0.00$) (Table 5).

5. Discussion

This study demonstrated that involving informal caregivers in a 7-week psychoeducational intervention program with clearly defined sessions, combining information and psychological support can have positive effects on the healthy life style behavior and caregiving burdens of caregivers. When the Healthy Life Style Behaviors of caregivers of dementia patients in the “*First You Should Get Stronger*” Program and control group were examined, it was observed that caregivers need support regarding health responsibility, physical activity, feeding, moral development, interpersonal communication and stress management sub-dimensions. Physical activity, balanced and regular diet, weight management, accordance to drug treatment, alcohol use, smoking and low stress are all related with physical health (Lucini, Zanuso, Blair, & Pagani, 2015; Olsen & Nesbitt, 2010). Healthy life style behaviors are shaped with the personal preferences of individuals; however it is insufficient for the health of individuals for it to remain only as a personal preference. It is important to provide information to the individuals for the long term effects of healthy life style behaviors (Carstensen, Rosenberger, Smith, & Modrek, 2015).

According to the data obtained in our study, information levels of the participants of the “*First You Should Get Stronger*” program increased regarding healthy life style behavior. Similar results have been obtained in studies during which the effects of intervention programs applied on caregivers of dementia patients have been examined. It was determined in the study carried out by Hébert et al. (2003) that improvements in the coping abilities of caregivers and important effects on the disruptive behaviors were observed as a result of the psychoeducation intervention applied on caregivers of dementia patients (Hébert et al., 2003). Alves et al. (2015) carried out a study in which similarly they indicated that psychoeducational intervention effectively helped caregivers to cope with their feelings and with their daily lives (Alves et al., 2015). It was determined in a study during which home-based caregiver education program was applied that family caregivers

who received the individualized home based education program had better health outcomes in bodily pain, role disability due to emotional problems, vitality, better mental summary score, and decreased risk for depression (Kuo et al., 2013). No intervention program was found in literature which evaluates the healthy life style behaviors of caregivers of dementia patients.

It has been considered in this study that the “*First You Should Get Stronger*” program will contribute to the relevant literature due to its positive effect on developing healthy life style behavior. In our study, burden levels for caregivers of dementia patients were observed to be high for both the intervention and the control groups prior to the “*First You Should Get Stronger*” Program. Papastavrou et al. (2007) carried out a study in which they determined that 65% of the caregivers are subject to high levels of burden and show depressive symptoms and that the burden experienced is related with the coping methods used (Papastavrou et al., 2007). Similarly, it was also put forth in another study that the physical and mental health of the caregivers of dementia patients decrease with increasing burden (Chen, Chen, & Chu, 2015). It is important that healthcare professionals evaluate the burden and coping with stress levels of caregivers and inform them with regard to managing the behavior of the patients as well as coping with their own emotions (Chen et al., 2015; Papastavrou et al., 2007).

It was determined as a result of our study that the burden levels of caregivers who participated in the “*First You Should Get Stronger*” Program were lower in comparison with those of the control group at a statistically significant level. Brodaty, Green, and Koschera (2003) carried out a meta-analysis study in which they evaluated the effects of intervention programs on caregivers of dementia patients and stated that the intervention programs are effective especially on their psychosocial stress, knowledge levels as well as the mood of the patient, however contrary to our findings, they also indicated that they are less effective on the burden of the caregivers (Brodaty et al., 2003). Martín-Carrasco, Domínguez-Panchón, González-Fraile, Muñoz-Hermoso, and Bal-lesteros (2014) carried out a study in which it was put forth that the psychoeducation intervention program applied on caregivers of dementia patients was not effective on the caregiving burden of the caregivers (Martín-Carrasco et al., 2014). Many studies have demonstrated benefits for strategies designed to relieve caregiver burden in dementia. Another study examining the intervention programs related with caregivers of dementia patients put forth that psychoeducational interventions have positive effects on the burdens of dementia patient caregivers (Carretero, Garcés, Ródenas, & Sanjosé, 2009). Savundranayagam, Montgomery, Kosloski, and Little, 2011 carried out a study in which it was put forth that the psychoeducation intervention applied during the study has a statistically significant effect on the objective burden of caregivers. The quality and wellstructuring of the psychosocial educational interventions related with caregivers of dementia patients are variables which are related with the burden that the caregiver experiences (Carretero et al., 2009).

Table 5
Zarit Caregiving Burden Scale Score Averages Comparison for the Experiment and Control Group.

Caregiving Burden	Experiment Group $\bar{x} \pm SS$		Control Group $\bar{x} \pm SS$		Before the Program	After the Program
	Before the Program	After the Program	Before the Program	After the Program	Test value, p	Test value, p
ZCBS Total Score	78.30 $\pm$ 4.46 $z: -5.434, p:0.00$	67.10 $\pm$ 7.18	78.45 $\pm$ 3.26 $z: -1.437, p:0.165$	79.30 $\pm$ 3.79	U:25.00, p:0.56	U:40.00, p:0.00

z :Wilcoxon Sign Test.

U:Mann Whitney U test.

It is stated that educational interventions applied via home visits to caregivers of dementia patients are more effective (Thinnes & Padilla, 2011). The “**First You Should Get Stronger**” program might have had a positive impact on the burden of caregivers since it is also a psychoeducation intervention applied via home visits. Despite the positive effects of intervention programs, a lack of standardization in study design, assessment of different outcomes and the use of a variety of different measurement tools make it difficult to judge which are the most effective.

5.1. Limitations

This study is limited with the caregivers of individuals with dementia registered at the Erenköy Rotary Club Family Health Center in the Kepez district of Antalya who are open to communication and who have agreed to participate in the study.

6. Conclusion

According to the results obtained from the study, it can be stated that the “**First You Should Get Stronger**” program has positive effects on the healthy life style behaviors and burden of caregivers of dementia patients. It is important for the healths of caregivers to apply the “**First You Should Get Stronger**” program with a larger sample group and that similar programs for caregivers of dementia patients are always included as part of continuous and regular applications. Dementia wears down caregivers significantly since it is a difficult neurological syndrome that is generally cared for in the house. It is also suggested that the nurses and other healthcare professionals working with dementia patients should be evaluated separately and that they should provide care as part of the “**First You Should Get Stronger**” program.

Conflicts of interest statement

The authors certify that they have NO affiliations with or involvement in any organization or entity with any financial interest, or non-financial interest in the subject matter or materials discussed in this manuscript.

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